

February 23, 2026

## **ELECTRONIC SUBMISSION**

Office of the Deputy Secretary and Assistant Secretary for Technology Policy (ASTP) and  
Office of the National Coordinator for Health Information Technology (ONC)  
Department of Health and Human Services (HHS)  
Attention: HHS Health Sector AI RFI  
Mary E. Switzer Building  
330 C Street SW  
Washington, DC 20201

## **RE: Request for Information: Accelerating the Adoption and Use of Artificial Intelligence as Part of Clinical Care**

To Whom It May Concern,

Medtronic is the world's leading medical technology company, specializing in implantable and interventional therapies that alleviate pain, restore health, and extend life. We are committed to the continual research and development necessary to produce high-quality products and innovative therapies that improve health outcomes for all patients. Medtronic's devices and therapies address over 70 chronic conditions and diseases, across four portfolios, including Cardiovascular, Medical Surgical, Neuroscience, and Diabetes.

As a medical technology developer, we applaud ASTP/ONC for issuing an RFI on actions the Department can take to establish a forward-leaning, industry-supportive, and secure approach to accelerate the adoption and use of AI as part of clinical care. HHS has a pivotal role to play in creating a cohesive regulatory, payment, and evaluation environment that encourages AI adoption while protecting patient safety and data privacy.

In particular, we appreciate this opportunity to provide input regarding how reimbursement approaches can be most effectively applied to support the rapid adoption and use of AI in clinical care, to reduce uncertainty that impedes AI innovation, and to align federal incentives so that AI is deployed in ways that enhance productivity, reduce burden, lower health care costs, and improve health outcomes for patients, caregivers, and communities. We also appreciate that the RFI raises additional questions, such as those related to regulatory, interoperability, and cybersecurity considerations. We have worked extensively with our trade associations, AdvaMed and MDMA, to provide additional feedback on these extremely important topics.

Specifically, we offer comments on the following questions raised in the RFI:

- Barriers to Private Sector Innovation in AI and Its Adoption and Use (Question 1)
- Payment Policy Changes to Incentivize the Effective Use of AI in Clinical Care (Question 2)
- Performance and Cost Expectations and Experiences in the Deployment of AI Tools (Question 6)

Our responses to each of these questions as they relate to reimbursement are outlined in detail below.

### **1. What are the biggest barriers to private sector innovation in AI for health care and its adoption and use in clinical care?**

The most significant barrier to AI innovation, adoption, and use in clinical care that we seek to highlight in this response is the lack of a clear, predictable reimbursement pathway under Medicare to account for the costs of these technologies. AI tools used in clinical care have the potential to reduce overall health care costs by improving clinical efficiency, supporting more timely and accurate clinical decision-making and enabling high quality care in the face of healthcare workforce shortages. Additionally, many AI tools used in clinical care improve patient outcomes through more accurate and earlier disease detection, and better treatment recommendations. At the same time, adoption of these technologies imposes real and often significant upfront and ongoing costs on hospitals and physician practices that are not adequately recognized under current reimbursement frameworks.

As HHS and CMS consider barriers to adoption of AI tools for clinical care, it cannot be understated the importance that adequate reimbursement will play to support future adoption. CMS has long recognized that improvements in efficiency mean less work for physicians and have accordingly adjusted Physician Fee Schedule payment to account for that work reduction. These increases in efficiency are due in part to improvements in technology including use of AI tools for clinical care; however, there is no corresponding payment to support adoption of these technologies by hospitals and physician practices. Under prevailing fee-for-service payment models, many of these savings accrue outside the provider setting that bears the cost of adopting and maintaining the AI technology.

For providers, implementation of AI tools used in clinical care requires meaningful investment, including but not limited to:

- Acquisition or subscription costs for AI software and/or enabled hardware;
- Integration with existing clinical IT systems, including EHRs, PACS, and device infrastructure;
- Cybersecurity, data governance, and compliance resources;
- Training of clinicians and staff and ongoing workflow optimization; and
- Maintenance, updates, validation, and performance monitoring over time.

These costs are incremental to existing operating expenses and are often incurred without a corresponding increase in reimbursable services. In some cases, AI may even reduce billable volume by preventing unnecessary interventions, creating a financial disincentive for adoption despite clear clinical and operational value. As a result, the current reimbursement environment may unintentionally slow adoption of high-value AI technologies, particularly for smaller hospitals, rural providers, and independent physician practices with limited capital and IT resources. Without targeted reimbursement pathways or value-based incentives, providers may be unable to justify investment in AI even when such tools demonstrably improve efficiency, safety, and patient outcomes.

Aligning reimbursement policy with the economic realities of AI implementation is essential to ensure that providers are not penalized for adopting technologies that improve care delivery and system-wide efficiency. Thoughtful reimbursement strategies can accelerate responsible AI adoption while promoting sustainability for hospitals and physician practices.

**2. What regulatory, payment policy, or programmatic design changes should HHS prioritize to incentivize the effective use of AI in clinical care and why? What HHS regulations, policies, or programs should be revisited to augment your ability to develop or use AI in clinical care? Please provide specific changes and applicable Code of Federal Regulations citations.**

As CMS further considers the establishment of a reimbursement framework for software as a service (SaaS), we believe that future policies should strike an appropriate balance, by easing financial barriers to hospital and physician adoption without providing incentives for over-utilization. In the context of Medicare's Outpatient Prospective Payment System (OPPS) for hospitals, CMS has relied upon temporary New Technology Ambulatory Payment Classifications (APCs) and transitional pass-through (TPT) payments for devices to provide this balance. However, there is currently no pathway for physicians to receive reimbursement for utilizing SaaS in their practices. We urge HHS to direct CMS to adapt these mechanisms to enable adoption of AI in clinical care in hospital outpatient, ambulatory surgical center (ASC) settings, and physician practices under fee-for-service Medicare, while in parallel pursuing value-based payment mechanisms for long-term sustainability, as outlined in detail below.

New Technology APCs

The use of procedural C-codes with payment established through New Technology APCs, enables patient access during the initial period of clinical adoption, while simultaneously facilitating the collection of hospital cost data to inform permanent APC assignments in the future. CMS could effectively utilize this approach for AI to increase hospitals' ability to invest in innovative technologies to advance the speed, accuracy, and quality of diagnostic and therapeutic decision making in health care, ultimately enabling improved standards of care.

More specifically, we support the payment pathway outlined in The Health Tech Investment Act (S. 1399) for algorithm-based healthcare services (ABHS) using New Technology APCs, which we believe CMS could adopt using existing authorities. The payment pathway outlined in this legislation provides a dedicated framework for Medicare to reimburse for ABHS, defined as services provided with an FDA approved or cleared medical device that utilizes AI, machine learning (ML), or other similar technology to produce clinical outputs or generate clinical conclusions with physician or other qualified health care professional involvement in the screening, detection, planning, diagnosis, or treatment of an individual's condition or disease. Under this pathway, CMS could adapt the New Technology APC application process

and criteria to ensure that all ABHS, including those that are performed in conjunction with an underlying service, are eligible for payment under the OPPTS. In addition, ABHS could be assigned to a New Technology APC for no less than five years until there is sufficient data on costs to assign the service to a clinical APC or create a new APC.

The New Technology APC pathway is also a suitable approach with respect to coding for ABHS, because it allows CMS to create procedural C-codes as needed and outside the American Medical Association (AMA) CPT Editorial Panel process. CPT codes are established by the AMA primarily to describe physician services, and therefore may not be sufficient for hospital and ASC billing purposes in all cases. For procedures involving sophisticated clinical software, including AI, acquired on a subscription or per-use basis, CPT codes may not reflect facilities' costs to perform the procedure, and therefore fail to comprehensively describe the service from the standpoint of the facility. In such cases, the creation of procedural C-codes by CMS may be necessary to facilitate appropriate billing and payment for ABHS by hospitals and ASCs.

In the absence of finalized legislation, we believe CMS should propose a unified and predictable reimbursement framework leveraging the New Technology APC pathway for software technologies based on CMS-defined categories and existing statutory authority, to help address the current uncertainty around valuation and to encourage consistent adoption of these technologies in hospital outpatient and ASC settings of care.

#### TPT Payment for Devices

Under the OPPTS, temporary TPT payments are used to enable hospital adoption of implantable medical devices that are new, costly, and that have either evidence demonstrating substantial clinical improvement compared to available treatments, or Breakthrough Device status as designated by the FDA. The current regulation at 42 CFR 419.66(b)(3) requires devices to come into contact with human tissue, and therefore excludes AI technologies. We urge CMS to consider adapting the TPT requirements, or consider developing a TPT-like framework, to accommodate high-value AI innovations that would otherwise not be accounted for under the cost-based prospective payment system methodology, in order to reduce hospital barriers to adoption.

### Physician Fee Schedule

At present, there are no established payment pathways for physicians to be reimbursed for the acquisition or use of SaaS solutions in the office setting. As medical technology advances, particularly in the area of implantable devices, physicians lack a predictable mechanism (and in some cases, any mechanism) to recover investments associated with these innovations. For example, emerging implantable devices are being designed with the capability for future FDA approved software “unlocks,” enabling enhanced therapeutic functionality over time. These new, enhanced software-based capabilities will allow patients and clinicians to benefit from technological advancements without requiring full device replacement, thereby eliminating clinical risks of reoperation, and improving outcomes while preserving the value of the original implant.

### Value-Based Payment

While improvements to Medicare fee-for-service reimbursement methodologies are vital in the short-term to ensure AI innovation in clinical care does not stagnate, we encourage HHS to maintain an eye toward a future in which payers’ and providers’ incentives are aligned to provide the best possible care to patients at the lowest possible cost. In doing so, CMS should explore how stronger incentives for high-value AI clinical interventions can be created through existing quality measurement programs and through CMMI payment models. For example, outcomes associated with the use of AI in clinical care could be studied and rewarded through a framework similar to that of the upcoming ACCESS model.

## **6. Where have AI tools deployed in clinical care met or exceeded performance and cost expectations and where have they fallen short? What kinds of novel AI tools would have the greatest potential to improve health care outcomes, give new insights on quality, and help reduce costs?**

As indicated above, AI tools used in clinical care can improve patient outcomes through more accurate and earlier disease detection, and better treatment recommendations. At the same time, adoption of these technologies imposes real and often significant upfront and ongoing costs on hospitals and physician practices that are not adequately recognized under current reimbursement frameworks. We offer two examples of AI technologies – AI-assisted

colonoscopy and fractional flow reserve angiography (FFRangio) – that exceed performance expectations yet face challenges in terms of their deployment in clinical care to illustrate the barriers that inadequate and unclear reimbursement pathways can present.

### AI-Assisted Colonoscopy

Medtronic's GI Genius™ CAdE Intelligent Endoscopy Unit enhances patient care through the use of AI to identify suspicious lesions in real time for evaluation during a colonoscopy, and is clinically proven to increase the adenoma detection rate (ADR). In a randomized controlled trial, the GI Genius™ system resulted in an average absolute ADR improvement of 14.4% compared with standard colonoscopy,<sup>1</sup> increasing the odds that patients with lesions will be detected and treated earlier. Importantly, each 1% improvement in ADR has been shown to lead to a 3% reduction in interval colorectal cancer over a 10-year follow-up period.<sup>2</sup> GI Genius™ has been further shown to reduce the Adenoma Miss Rate.<sup>3</sup> Despite the tremendous benefit the use of GI Genius™ can provide – both to individual patients and at a population health level – there are currently no coding and payment mechanisms to account for the costs of facilities and physicians adopting this technology.

### FFRangio

Evidence shows that when fractional flow reserve (FFR) is used to help classify coronary lesions there is a significant reduction in stenting, lower costs, and improved outcomes.<sup>4,5</sup> Traditional FFR is a wire-based procedure, but FFRangio provides a less invasive method of evaluating hemodynamic relevance of coronary artery stenosis by using AI analysis of routine angiography images to create three-dimensional functional models of the coronary tree. FFRangio is shown to have a comparably high degree of diagnostic accuracy to wire-based

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<sup>1</sup> Repici, Alessandro et al. "Efficacy of Real-Time Computer-Aided Detection of Colorectal Neoplasia in a Randomized Trial." *Gastroenterology* vol. 159,2 (2020): 512-520.e7. doi:10.1053/j.gastro.2020.04.062.

<sup>2</sup> Corley, Douglas A et al. "Adenoma detection rate and risk of colorectal cancer and death." *The New England journal of medicine* vol. 370,14 (2014): 1298-306. doi:10.1056/NEJMoa1309086.

<sup>3</sup> Wallace, Michael B et al. "Impact of Artificial Intelligence on Miss Rate of Colorectal Neoplasia." *Gastroenterology*, S0016-5085(22)00238-4. 11 Mar. 2022, doi:10.1053/j.gastro.2022.03.007.

<sup>4</sup> Tonino PAL, De Bruyne B, Pijls NHJ, et al. Fractional Flow Reserve versus Angiography for Guiding Percutaneous Coronary Intervention. *N Engl J Med*. 2009;360(3):213-224.

<sup>5</sup> Fearon WF, Yeung AC, Lee DP, et al. Economic Evaluation of Fractional Flow Reserve-Guided Percutaneous Coronary Intervention in Patients With Multivessel Disease. *Circulation*. 2010;122(24):2545-2550.

FFR and is cost-saving,<sup>6,7</sup> but adoption of the technology has been slow because of the complexity and uncertainty of existing reimbursement pathways and lack of adequate hospital payment. Perhaps most troubling, because wire-based FFR is more expensive, hospitals often receive higher payments under the OPPS relative to FFRangio, which may unintentionally incentivize hospitals to maintain the status quo of utilizing a more invasive analog procedure over an AI-based technology which would achieve comparable results at a lower cost.

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We appreciate the opportunity to provide input on accelerating the adoption of AI in clinical care. Thoughtful and comprehensive approaches to ensure hospitals, ASCs, and physicians are adequately reimbursed for use of these technologies will encourage adoption, reduce uncertainty and speed innovation. Thank you for your consideration of our input on these important questions and we look forward to further engagement with HHS. If you have questions or need further information, please contact me at (202) 441-1938 or [carrie.w.bullock@medtronic.com](mailto:carrie.w.bullock@medtronic.com).

Regards,



Carrie Bullock

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<sup>6</sup> Witberg G, et al. Diagnostic performance of angiogram derived fractional flow reserve. J Am Coll Cardiol Interv. 2020 Feb, 13 (4)

<sup>7</sup> <https://www.prnewswire.com/apac/news-releases/provision-study-first-ever-prospective-rct-of-cathworks-ffrangio-outcomes-compared-to-invasive-wire-based-physiology-in-japan-meets-its-primary-endpoint-302288035.html>.